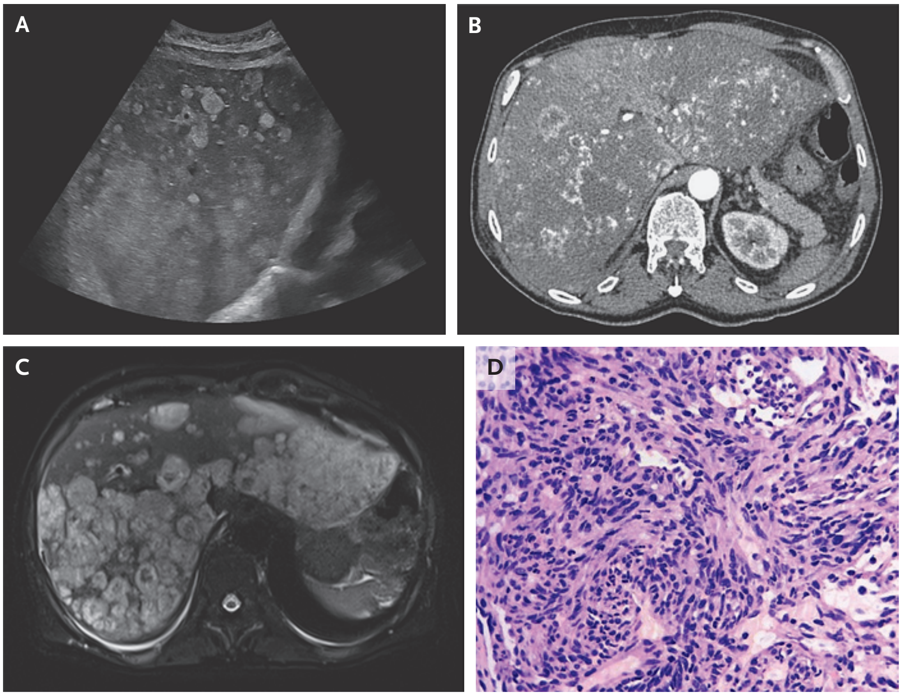
IMAGES IN CLINICAL MEDICINE

Hepatic Angiosarcoma



A 60-YEAR-OLD MAN PRESENTED WITH A 15-DAY HISTORY OF GENERALIZED weakness and right upper abdominal pain. A physical examination was notable for tender hepatomegaly. Laboratory testing showed mild elevations in aspartate aminotransferase and alanine aminotransferase levels. Abdominal ultrasonography revealed an enlarged liver with multiple hyperechoic nodules, resulting in diffuse disruption of the hepatic architecture (Panel A); these findings suggested an infiltrative vascular process. Computed tomography of the abdomen, performed after the administration of intravenous contrast material, revealed innumerable heterogeneous nodules and areas of necrosis and hemorrhage, which produced a mosaic pattern of enhancement (Panel B, arterial phase, axial view). Magnetic resonance imaging of the liver revealed multiple lesions with high signal intensity on the T2-weighted sequence (Panel C, axial view), a finding consistent with a vascular neoplasm. Histopathological examination of a needle-biopsy specimen of the liver showed proliferation of vasoformative and spindle-shaped cells (Panel D, hematoxylin and eosin stain). Immunohistochemical testing was strongly positive for endothelial markers ERG and CD31. A diagnosis of hepatic angiosarcoma — a rare, highly aggressive primary malignant vascular tumor of the liver — was made. Hepatic angiosarcoma is associated with exposure to vinyl chloride, arsenic, or anabolic steroids, which this patient did not have. One month after diagnosis, the patient died from rapidly progressive liver failure.

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